

SUMMARY RECORD OF THE FIFTH MEETING

(Putrajaya International Convention Centre, Unity Hall A2,
and at Unity Hall A1, Concourse Level)
Wednesday, 13 October 2010, at 14:00

CHAIRPERSON: Ms Amenta Matthew (Marshall Islands)

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1. REGIONAL STRATEGY TO STOP TUBERCULOSIS 2011–2015: Item 13 of the Agenda
(document WPR/RC61/8) (continued)

Dr VILLAYERDE (Philippines) praised the Regional Strategy, which focused on care for the vulnerable and marginalized, and management of MDR-TB and XDR-TB; he proposed the following amendments. Under objective 1, universal and equitable diagnosis and treatment, a multisectoral approach to TB control should be adopted, with care providers from public and private sectors working in partnership. For objective 2, strengthening laboratory capacity, WHO should ensure that new technologies were made available and affordable to all, through national networks of laboratories. In objective 3, scaling up programmatic management, the experience of countries that had successfully scaled up management of MDR/XDR-TB should be tapped through public-private partnership. For objective 5, strengthening TB programme management, regulatory mechanisms for rational use of TB drugs should be devised.

Ms TOELUPE (Samoa) on behalf of her country, endorsed the Regional Strategy, noting its allowance for diversity of cultures. objective 3, scaling up programmatic response to MDR-TB and XDR-TB, was a major concern in Samoa. Grant application requirements of agencies such as the Global Fund to Fight AIDS, Tuberculosis and Malaria were tedious, and placed heavy demands on fragile programmes. Samoa sought technical and financial assistance for all objectives, to make elimination achievable.

Dr XIA (China) agreed with the analysis of existing problems, and with the five core objectives of the Strategy, which would help countries to draft their national plans, as China was doing at present: national objectives were all in line with the Western Pacific Regional Strategy. His Government would actively respond to requirements of the Regional Office, increasing investment in order to improve managerial capacity and provide equitable access to treatment, care and services. China would strive for a major breakthrough in preventing and treating XDR-TB and the TB-HIV co-infection.

Dr KHALIFAH ISMAIL (Brunei Darussalam) noted the importance of TB control, especially as regards MDR-TB and the TB-HIV co-infection. The DOTS strategy was being maintained in his country, with a high cure rate of 95%, but the challenge remained. Elements of the Regional Strategy, such as case detection, had to be strengthened. Brunei therefore supported the Strategy and looked forward to further work with WHO in capacity building and sharing of experience.

Dr SHARMA (Fiji) endorsed the Regional Strategy, which was associated with Millennium Development Goal 6. HIV prevalence in TB patients was 11% in Fiji. TB was being treated with delays in case detection and screening and 2–3 months of inpatient care followed by DOTS, up to a total of six months. There were financial problems, but now, with support from the Global Fund, the situation should improve. Co-infection seemed to threaten both the HIV and TB programmes, so guidelines on patient management were being designed.

Mr KAHU (Vanuatu) reported no evidence of a fall in TB morbidity in recent years in his country, but rather an increase, perhaps thanks to active case detection. Rates were low, but there was not a reliable health information system. Major problems included inadequate staff, weak case detection, and a lack of outreach work with high-risk and vulnerable groups to encourage them to seek early diagnosis and treatment. Vanuatu planned to expand its programme with help from the Global Fund, WHO and other partners. There would be more collaboration with other programmes such as HIV/AIDS, even although HIV prevalence was low.

Malaria and TB microscopists would be trained in reading smears for both diseases; this would benefit both programmes. Vanuatu was now updating its treatment guidelines, and would put the Regional Strategy into practice.

Dr SONG (Republic of Korea) congratulated Member States and the Western Pacific Region on the likely success in halving TB prevalence and mortality between 2000 and 2010. That could not have been achieved without great effort, based on the two previous strategies. However, failure to detect all cases, to ensure fair access to good treatment and to deal with MDR-TB and XDR-TB made further progress difficult. The new Regional Strategy was therefore timely, and the Republic of Korea endorsed it, acknowledging the Regional Office's work. It would continue to collaborate with other countries and with the Regional Office for better TB control.

Dr KUARTEI (Palau) remarked that the geographical location of Palau and the proximity of MDR-TB required trans-border monitoring to reduce the danger. The document mentioned co-morbidity of TB and diabetes in passing, but that problem ought to be covered, in view of growing evidence of the problem.

Dr MALAU (Papua New Guinea) acknowledged that the TB-HIV co-infection, and treatment compliance, including treatment of MDR-TB, were major challenges, given his country's fragile system and drug supply. With support from the Global Fund some progress had been made in seven of the 20 provinces, but national TB programme management should be based on technical and epidemiological evidence. Global Fund arrangements at present made it difficult to ensure that technical programmes at the country level addressed national programme priorities. Funding for critical programme areas was not equitably distributed. The national TB programme should be reviewed to ensure it dealt with national problems. He asked WHO to facilitate an independent review of TB programme funding initiatives. He endorsed the Strategy and looked forward to working on it with partner agencies.

Ms LANGIDRIK (Marshall Islands) reported that TB prevalence in her country was very high, and that the first case of MDR-TB had been reported in 2004. In 2009, with the assistance of WHO, the Secretariat of the Pacific Community and the United States Centers for Disease Control and Prevention, the programme had been strengthened. With support from the Government, the Global Fund and United States Compact funds, the Ministry of Health had recruited additional staff for the programme, and strengthened DOTS activities and contact tracing. There were two cases of MDR-TB in the Marshall Islands. Some health workers had been identified as contacts and were undergoing preventive treatment. The country had learnt the hard way, and thanked WHO for its technical assistance. The five objectives of the Regional Strategy were realistic. However, experience in her country showed that stigma was still powerful, preventing some from seeking treatment for themselves or their families. How should health services deal with patients who refused treatment for TB or leprosy? Another problem was migration and mobility of families from one island to another; if programme staff were not informed, treatment was interrupted, with consequent danger of drug resistance. The Marshall Islands supported the Regional Strategy, and looked forward to continuing support from WHO and regional partners.

Ms HSU (United States of America) said that infectious disease control was important to her country, which supported the Regional Strategy with its focus on surveillance, detection and laboratory diagnostic capacity, developing a comprehensive response to MDR-TB and preventing drug resistance. In March 2010, the United States Government had unveiled its TB strategy, which aimed to reduce the disease burden by

50% from its 1990 baseline, detecting at least 70% of sputum-positive cases, and treating at least 85% of cases in countries with established United States Government TB programmes. In that way, 2.6 million sputum-positive cases would be successfully treated with the DOTS programme by 2014, diagnosing and initiating treatment of at least 52 700 new cases of MDR-TB by the same year, with additional treatment through coordinated multilateral efforts. The United States was committed to working with WHO, the Western Pacific Region and Member States, to meet international standards, which meant extending the reach of TB care through DOTS and treatment of XDR-TB and the TB-HIV co-infection. Investment in research for new drugs, diagnostics and vaccines, and research to improve and measure results were urgently needed.

Dr TSOLMON (Mongolia) observed that TB was a major public health problem in his country, which had the seventh highest TB case rate among the 37 countries and areas in the Western Pacific Region. TB ranked sixth among causes of death for all ages in Mongolia. In the previous five years there had been some 4000 cases in the country, 2000 of them smear-positive. However, the country had reached the WHO target, since case detection and treatment success rates were now 83.5% and 89%, respectively. According to WHO estimates, TB prevalence was now 234 per 100 000 population, and mortality in 2007 was 29 per 100 000—less than in the year 2000. The Government had increased funding and, since 2007, had covered the cost of first-line TB drugs. Initiatives had been launched to cover prisoners and the homeless. In 2009, a national strategic plan had been approved. To stop TB, MDR-TB management had to be scaled up. The Government of Mongolia expressed its appreciation for all international assistance, especially from the Global Fund.

Dr CUTTER (Singapore) said that Singapore saw TB as a major public health problem that needed more attention. While strongly endorsing the Plan, he highlighted a practical problem that had emerged since publication of the third edition of the WHO publication *Tuberculosis and air travel: guidelines for prevention and control*, which recommended that “people with infectious or potentially infectious TB should not travel by commercial air transportation on a flight of any duration”. The same guidelines stated that patients with MDR-TB and XDR-TB should be considered non-infectious only after evidence of a clinical response to treatment and two consecutive negative sputum culture results had been obtained. However, that could take five or six months – and indeed visitors had been held in Singapore for such periods so that the authorities could enforce treatment for their MDR-TB.

However, a recent article in *The Lancet Infectious Diseases* by Dr Ibrahim Abubakar had made the following points. There was very limited evidence of transmission of TB during air travel; indeed, no case of active TB resulting from transmission during air travel had yet been reported. Three negative sputum smear samples on separate days in people on effective TB treatment suggested extremely low potential for transmission. Although an airline cabin was a closed, confined space, exposure was relatively short in comparison with household contact, or even to other modes of transport, where people might travel on the same routes daily. Most aircraft used for long-haul flights had good filtering of particles over 3 microns, effectively blocking the TB bacillus. Finally, aircraft cabins changed the air more than 15 times per hour, which was more than in negative-pressure isolations rooms for MDR-TB cases. He therefore asked WHO to review its guidelines.

Dr CHOU (Macao [China]) held that the Strategy would provide good guidance. TB in Macao had diminished, with a cure rate of over 95%, and in recent years a lot of screening had been provided for high-risk populations. Laboratory data were used for prevention and treatment, and the training of technical staff

had been intensified, in the interests of early detection and sound treatment. Close monitoring of private clinics was needed, to ensure that all cases got proper treatment, in order to reduce the risk of MDR-TB and XDR-TB.

Dr TENAUTA (Kiribati) said that the rate of notification of TB cases in his country was one of the highest in the Pacific region. He welcomed the positioning of health systems strengthening at the centre of the new regional TB control strategy. Support from various partners had made it possible to recruit community DOTS workers and retired health workers to supplement the country's small, overburdened health workforce and to provide technical guidance and training; to construct a complex housing an outpatient TB clinic and a modern TB laboratory, including culture facilities; and to obtain the necessary supply of drugs. A study was being conducted with partners to gain a clear understanding of the association between diabetes and TB, which would be the first such study in the Pacific. Kiribati was attempting to involve the community in running the TB control programme, with a coordinating committee made up of church leaders. With extensive support from outside sources, TB control was the best-funded health programme in the country.

Commenting that many of the preventable causes of TB lay outside the health sector, including overcrowding, poorly ventilated houses and poor urban planning, he said that those socioeconomic determinants should be emphasized in the new strategy. His country supported and endorsed the Regional Strategy.

Mr NGUYEN MANH CUONG (Viet Nam) said that the TB control programme in his country had been successful, but was now encountering problems associated with MDR-TB, the TB-HIV co-infection and integration of new techniques into all laboratories in its network. The proposed strategy addressed all those problems. The goals of his country's strategic plan for 2011–2015 were to halve the estimated prevalence of TB in 2000 and to find no increase in the rate of MDR-TB. In May 2010, his country had hosted the 18th Stop TB partnership meeting, where the successes of its TB control programme had been recognized.

Ms GOODSPEED (Australia) said that her country endorsed the new Regional Strategy, which acknowledged the new challenges to TB control and described the opportunities for interventions and new techniques that would change TB control in the Region. Continuing progress was being made in reducing TB prevalence and mortality, with expanding use of DOTS and successful treatment. Stronger links were needed between services for TB and for HIV infection, as appropriate in local contexts. Her Government welcomed the inclusion of measures to diagnose and treat high-risk populations such as prisoners and drug addicts, which would provide another opportunity to create a link with HIV services.

The REGIONAL ADVISER, STOP TB AND LEPROSY ELIMINATION, replying to the comments of Member States, said that a multisectoral approach to TB control would present opportunities for other sectors; for instance, most of the revolutionary diagnostic tools and laboratory networks established for TB could be used for other diseases. She agreed with a number of speakers that a public-private mix would be useful for diagnosis, as most TB patients first presented privately; however, efforts must be made to ensure that the proper guidelines and drugs were used. Regarding the difficulty of obtaining second-line drugs for MDR-TB, especially in countries where there were only a few cases, pooled procurement through the Global Fund would be considered, with a virtual stock maintained in the Philippines. The Regional Office was ready to provide technical assistance for each MDR case diagnosed in Member States. She agreed with the remarks of representatives regarding the importance of drug regulation, especially for the prevention of MDR-TB.

The link with diabetes mentioned by some representatives was being followed up in overall collaboration with colleagues working on noncommunicable diseases, including tobacco-related conditions. In response to the comment by the Representative of Singapore concerning the special measures to be taken for patients with MDR-TB, including preventing them from boarding airplanes, she said that the topic was under discussion at WHO headquarters. The 2003 guidelines were not currently being revised.

The DIRECTOR, COMBATING COMMUNICABLE DISEASES, welcomed the progress made in the Region, most of which had acknowledged the support from the Global Fund, the governments of Australia and New Zealand and WHO. The challenges that had been described in relation to MDR-TB, the TB-HIV co-infection and the link with diabetes were reflected in the new Regional Strategy. He was pleased to learn that national plans appeared to be aligned with the regional plan, with country-specific adaptation.

The REGIONAL DIRECTOR thanked Member States for their firm commitment to TB control. Their hard work and collaboration had resulted in remarkable achievements over the past 10 years. The emergence of MDR-TB threatened to set back much of the progress made, and the new strategy addressed that possibility. The availability of new diagnostic techniques, which would shorten the delay between testing and result from one to two months to 90 minutes, would have a huge impact on TB treatment and management. Nevertheless, the early detection of more cases of MDR-TB, in both developed and developing countries in the Region, would mean that more resources would have to be found to treat them. In many developed countries, it had been assumed that TB was under control, and it had been given low priority; that had led to its reemergence and to MDR forms. WHO was ready to support countries in facing new situations and seizing new opportunities, and to help them build on the progress made, with lasting results.

In the absence of further comments, the CHAIRPERSON asked the Rapporteurs to prepare a draft resolution to be considered later in the session.

2. ASIA PACIFIC STRATEGY FOR EMERGING DISEASES AND THE INTERNATIONAL HEALTH REGULATIONS (2005): Item 14 of the Agenda (document WPR/RC61/9)

The DIRECTOR, PROGRAMME MANAGEMENT, introducing the item, reminded the Committee that they had endorsed the first Asia Pacific Strategy for Emerging Diseases (APSED) in September 2005, and that it was now approaching the end of its successful implementation. APSED had been used extensively by WHO and Member States to strengthen capacity for managing emerging diseases, improving preparedness for pandemic influenza and complying with the requirements of the International Health Regulations (IHR 2005) for core capacity.

As the original APSED neared the end of its planned lifespan in 2010, WHO had consulted with Member States on areas in which successes had been achieved and areas in which improvements could be made. In addition, APSED had been put to the test during the H1N1 pandemic, and the lessons learnt during the pandemic had been taken into account in designing the new APSED. The new strategy was much broader. It still had its biregional feature, and it addressed not only emerging infectious diseases but also other acute public health emergencies.

The Committee was invited to review and consider endorsing APSED (2010).

Dr TORA (Fiji) said that implementation of APSED (2005) had saved many lives and resources in his country and improved collaboration among sectors. Furthermore, it had allowed Fiji to deal with pandemic influenza H1N1 (2009) effectively through sentinel surveillance and technical capacity and advice. Clinical practice guidelines had been distributed within 10 days, to inform health staff about the pandemic and give them confidence to manage it. He therefore welcomed the new APSED. He thanked the Regional Director for having mentioned lymphatic filariasis during the first meeting, as that disease was still a problem in Fiji, and he thanked WHO for its improved TB control strategy and for its programme for the surgical removal of hydrocoele.

Dr JACOBS (New Zealand) said that emerging diseases remained a concern for all the countries of the Region. APSED (2010) would provide a useful framework for strengthening health infrastructure for planning for and responding to emerging diseases, in compliance with IHR (2005), and would also be useful for dealing effectively with endemic diseases and with other health threats. His Government was pleased to see that the strategy addressed a number of fundamental issues. Risk communication was essential in addressing public health threats; in order for that capacity to be sustainable, it should be recognized as important, not only during emergencies, but also in daily public health work. Common, coordinated approaches were needed for health emergency planning, as opposed to separate, variable, disease-specific plans. Where possible, plans for emergencies should follow the standard approach for non-health plans in each country in order to facilitate work across sectors. As his was an island country, the Government was pleased that the strategy emphasized the importance of planning for responses at points of entry.

Dr PHILAVONG (Lao People's Democratic Republic) said that discussions at a national workshop on APSED and IHR (2005) held earlier in 2010 in his country showed that considerable progress had been made in the establishment of rapid response teams, field epidemiology training, strengthened laboratory capacity for influenza, human and animal health interfaces, the establishment of infection control measures and training in risk communication. A number of gaps remained, however, because of the short period of implementation of the national plan and low baseline capacity; implementation would continue in the coming two to three years. APSED (2005) had provided a useful framework for coordinating various capacity-building activities and for identifying gaps in capacity for dealing with emerging infectious diseases. He proposed that additional areas for capacity-building be addressed in future, including clinical management, food safety, chemical accidents, natural disasters and mass gatherings.

His country was fully committed to implementing the IHR (2005) but considered that the tool for monitoring core capacity should be simplified and harmonized with other assessment tools. He welcomed the goal of building sustainable national and regional capacity and the inclusion of new areas for capacity-building: public health emergency preparedness; regional preparedness; alert and response; and monitoring and evaluation. His country endorsed APSED (2010).

Dr MARGHEM (France) recalled that the health crisis associated with pandemic influenza H1N1 (2009) had shown that APSED should be more flexible and adaptable to different etiological situations. A structured, prepared response was essential for successful action. The regional strategy should focus on strengthening national capacity for health emergency preparedness. The French Pacific territories had a

specific problem with respect to implementing IHR (2005) because they were not Member States of WHO and could therefore not be designated as focal points. The resulting delay in the transfer of information to and from WHO would be deleterious to efforts to mount a rapid, effective response.

Dr KHALIFA ISMAIL (Brunei Darussalam) said that the recent pandemic had shown the necessity of remaining vigilant and being prepared for the emergence of new and previously controlled communicable diseases. The IHR (2005) did not only cover communicable diseases but also had a vital role to play in protecting global public health. Core capacities should be strengthened, including for control at points of entry, and he asked WHO for technical assistance in reviewing and assessing that capacity in his country. The updated APSED would assist Member States in meeting the core capacity requirements of IHR (2005), in order to ensure national, regional and global health stability and security. His country endorsed APSED (2010).

Dr LIANG (China) commended the Regional Office's work in the prevention and control of emerging diseases, the management of other public health emergencies and implementation of IHR (2005). With regard to APSED (2010), countries in the Region should strengthen their cooperation within the framework of IHR, so that they could respond collectively to emerging diseases and other public health emergencies. The new strategy represented a guide for strengthening the prevention and control of emerging diseases and other health emergencies, within national programmes. The resolution on APSED (2010) should make clear that the Regional Office would solicit and respect the opinions of Member States when making the plan for implementation of specific measures in the strategy. It was essential to strengthen systems for the control and management of infectious disease outbreaks, in order to safeguard public health. His country would continue to respond effectively to public health emergencies, in cooperation with other countries in the Region.

Dr VILLAYERDE (Philippines) supported the implementation of APSED (2010). The existing biregional strategy had proved to be a useful tool for weighing national priorities for building the core capacities required under IHR (2005) and had facilitated national planning across sectors for the five focus areas: surveillance; risk assessment and response; laboratories; zoonoses; infection prevention and control; and risk communications.

He highlighted the need for regional collaboration in information systems, particularly for surveillance data. He supported the provisions regarding the role of national IHR focal points, strengthening oversight and implementation at country level, and identification of a national coordinator and an oversight committee, with the participation of a senior public official.

He made the following recommendations for improving the draft 2010 Strategy. Under Monitoring and Evaluation (section 3.8), the use of the recommendations of the Technical Advisory Group and similar bodies should be institutionalized as benchmarks for assessing progress in implementation. Section 5 on implementation should contain a separate subsection on human resource requirements for implementation at country and local levels. Preparedness and response meant nothing if such requirements had not been anticipated and provided for, in particular during outbreaks, when human resources were needed for surge capacity. Sustaining efforts to both implement the draft Strategy and meet the requirements of IHR (2005) would need continued technical and financial assistance.

Dr NASERI (Samoa) acknowledged the need to align his country's strategy with the broader regional one and commended the Regional Office's work in updating APSED (2005). He also acknowledged the support

to Samoa for implementing IHR (2005) with work on a more transparent and accountable surveillance system for emerging and re-emerging diseases and public health events of international concern. He supported the revised draft biregional strategy.

The pandemic influenza H1N1 (2009) had been a valuable lesson that had helped to improve sentinel surveillance and responses. Although his Government had had issues with subregional guidance, the guidelines were helpful and WHO's position, conveyed by PacNet, was instrumental in determining the country's response. That response had been dictated by the lesson of history and justified on one principle: the need to err on the side of caution. One quarter of Samoa's population had died during the 1918 pandemic of Spanish influenza. That fact had determined the response and the Government was not guilty of over-reacting. Had the H1N1 (2009) pandemic not turned out to be mild, the results could have been much worse, as Samoa did not have the resources or capacity to cope.

Dr VU SINH NAM (Viet Nam) expressed appreciation of the work of WHO to update APSED (2005) and to strengthen national and regional capacities, including meeting the core requirements of IHR (2005). APSED (2005) had been useful for Member States in several ways, from help in the setting of priorities and capacity building to improving resource mobilization. Considerable progress had been made in the areas of surveillance and response, laboratory capacity, collaboration between the animal and human health sectors, infection control and risk communication. Event-based surveillance in many countries had detected a wide range of emerging disease; trained rapid-response teams were in place to investigate outbreaks; and field epidemiology training programmes had been established in his and other countries in the Region. Viet Nam had used APSED (2005) to improve its surveillance system, laboratory capacity, multisectoral collaboration and infection control. His Government had shown its commitment in successfully containing epidemics of severe acute respiratory syndrome (SARS) and influenza (H5N1 and H1N1) and improved its preparedness for other emerging and re-emerging diseases, such as cholera and dengue.

APSED (2010) would facilitate the building of sustainable national and regional capacities and partnerships to cope with epidemics and other public health emergencies. He endorsed the draft revised strategy. His Government was committed to implementing it and sharing experiences with support from WHO.

Dr CUTTER (Singapore) underlined the importance of continued improvement in capabilities and capacities for surveillance and response to emerging diseases and other public health emergencies. There was always room for improvement and he endorsed the draft APSED (2010), which provided valuable guidance.

Mr SIGOTO (Solomon Islands) recognizing the importance of preparedness for pandemics, said that the pandemic influenza H1N1 (2009) must not be left to pass without the means being put in place to enhance the capacity to respond to future outbreaks. In the recent reorganization of the Ministry of Health in his country, a division dedicated to disease surveillance and pandemic response had been created. The Ministry was also organizing a workshop with WHO and the Secretariat of the Pacific Community on emerging diseases.

Dr NISHIZWA (Japan), noting the value of the biregional strategy during the pandemic influenza H1N1 (2009), endorsed APSED (2010). She expressed the hope that cooperation with the WHO South-East Asia Region would continue and recommended that the Regional Office should approach its counterpart in order to establish a framework for further cooperation.

The draft APSED (2010) went further than its predecessor, going from “establishing ad hoc capacities” to “measures with long-term sustainability”. She drew attention, however, to the wide variations in the extent of information on influenza H1N1 issued by national IHR focal points, which raised concerns about the control of the quality of data. Sometimes, the question of whether to issue particular data was difficult: it was unclear to Member States how to judge whether and what to send. The Secretariat should investigate and provide further guidance. Further, some Member States lacked even core capacities. Implementing APSED (2010) would depend on raising the minimum level and strengthening capacity. The Regional Office should survey the status of core capacities and report the findings so that WHO and partners could give priority to those countries that lagged behind. In terms of crisis management, correct information should be shared. The Regional Office should facilitate the dissemination of more information, including translation of crucial technical documents into the major languages of the Region.

Dr YUAN (United States of America), noting the high priority her country gave to prevention and control of infectious diseases and its work in the Region to reduce the risk of emerging infectious diseases, strongly supported the development of the draft revised strategy. The recent pandemic influenza H1N1 (2009) had underlined the need for better surveillance and reporting, improved laboratory capacity, and coordination between Member States and multilateral organizations. It was vital for Member States to cooperate in an open, transparent manner in the event of future outbreaks of emerging diseases.

APSED (2010) was wide-ranging and incorporated the lessons learnt from both implementation of APSED (2005) and the recent pandemic influenza H1N1, but could be strengthened with two changes. The strategy should explicitly mention the need to strengthen laboratory-based surveillance of sexually transmitted infections, in particular those due to highly-resistant strains of *Neisseria gonorrhoeae* that were emerging in the Region. Objectives 3 and 4, dealing with preparedness and early response, should be clarified. Would they cover the provision of second-line medicines for tuberculosis, which currently take months to deliver after diagnosis of a case of multidrug-resistant disease?

She stressed that all States Parties should fulfil their obligations under IHR (2005) in the implementation of APSED (2010). Those obligations included open and transparent sharing of information and ensuring that health measures were commensurate with public health risks and avoided unnecessary interference with international traffic and trade. WHO should work with States Parties to identify needs in resource-constrained countries, work with potential donors and find innovative solutions. The IHR monitoring framework for core capacity development was useful for States Parties to assess progress in implementation of IHR (2005) and for WHO to monitor that progress and identify outstanding needs.

Ms BENNETT (Australia) commended the efforts of Member States, WHO and development partners in implementing both APSED (2005) and IHR (2005). Significant progress had been made under APSED (2005), particularly in surveillance and response. She endorsed the revised strategy, APSED (2010), commending the work of the Regional Office. Implementation would need several actions. The Secretariat would need to work with national governments and other partners to ensure that realistic targets were set and appropriate resources were available for activities against emerging diseases. Strong planning, prioritization and resource mobilization were needed at country level. The strategy should be integrated, where possible, with related activities, such as infectious disease control. A robust monitoring and evaluation framework should be set up, integrating gender concerns and adapted to the needs of Pacific island and other small countries.

A strong regional capacity was the most effective and appropriate response for individual countries to protect citizens against emerging infectious diseases. Australia's experience of pandemic influenza and SARS indicated the vulnerability of all countries in the Region and the need for collaboration in prevention and response to emerging infectious diseases and other public health emergencies.

Dr LAM (Macao [China]) said that Macao had experienced outbreaks of emerging diseases and that APSED (2005) had provided a good framework for preparing for and responding to the pandemic influenza H1N1 (2009). The revised draft strategy gave a more comprehensive framework for similar activities relating to emerging infectious diseases, and Macao had already established its mechanism to improve the response to public health emergencies in line with it. He endorsed APSED (2010). Macao would draw up action plans and was committed to multisectoral implementation of the revised strategy.

Dr LEE (Republic of Korea) paid tribute to the Regional Office's rapid and timely response during the recent pandemic influenza H1N1 (2009), noting that the Region's achievements were realized within the framework of APSED (2005) and IHR (2005). The lessons learnt would provide perspective for future challenges but, as eradication of various diseases neared, complacency must be avoided and work done to optimize the valuable tool. For that reason, revision of APSED (2005) had been opportune and the revised strategy would continue to provide an essential platform for the strengthening of national capacity to respond to public health threats. Nevertheless, weaknesses in the Strategy had to be identified and redressed.

The Regional Office should direct its efforts towards strengthening the capacity of Member States, especially those with constrained resources, to implement the core components of APSED (2010), in particular providing legal, technical and financial support. Member States should work towards incorporating the major components of APSED (2010) into their legal frameworks and harmonizing national legislation with the provisions of the Strategy, and engage all stakeholders in the capacity-building process. He expressed confidence that the Regional Office and Member States would resolve the issues mentioned and endorsed APSED (2010).

Mrs PEARCE (Tokelau) recommended that the draft strategy should contain indicators on the inclusion of APSED (2010) in national emergency or strategic plans. Small countries like hers had limited capacity to keep abreast of the many plans that were issued by WHO, and her Government had opted to integrate several into one plan in order to facilitate implementation at both national and local levels.

She asked that the text contain post-event and/or post-recovery strategies, especially following natural disasters. Communities wanted to know how their lives would be restored. Review of the events during the recent pandemic and disasters with public health consequences indicated the need for better collaboration, communication, coordination and cohesion. She also asked that the plan be kept simple and flexible so as to allow countries to weave their own ways of working into it. Procurement of pharmaceuticals was straightforward, but her country needed logistical planning, for instance to cope with the absence of a cold chain – her country had no airport and a ship arrived every two weeks. Laboratory facilities suffered for the same reasons and rapid influenza test kits had proved to be useless during the recent pandemic. She endorsed the draft APSED (2010) and looked forward to support for integrating it into existing plans.

Dr 'AKAU'OLA (Tonga) echoed support for APSED (2010), adding that Tonga had benefited from its predecessor, which had guided its work on mitigating, preparing for and responding to emerging diseases

and especially pandemic influenza H1N1 (2009). The revised Strategy offered opportunities to strengthen preparedness further in anticipation of a future epidemic or pandemic.

He applauded WHO and its scientific partners for having produced a vaccine within six months of the outbreak of pandemic influenza. That achievement provided reassurance for vulnerable countries for the future.

Dr TSANG (Hong Kong [China]) said that APSED (2005) had proven to be extremely versatile for the response of Hong Kong to pandemic influenza H1N1 (2009) and in strengthening core capacities as required by IHR (2005). He fully supported and endorsed the expanded scope and focus areas of APSED (2010), which would enhance the ability of public health bodies to respond to major public health emergencies.

He asked whether consideration had been given to the name APSED as the 2010 strategy was no longer limited to emerging diseases.

The DIRECTOR, HEALTH SECURITY AND EMERGENCIES, welcomed the strong support for APSED (2010). In reply to comments, he said that the importance of monitoring and evaluation and the need to simplify reporting had been raised during the consultative process. Monitoring and evaluation had been incorporated into APSED (2010) but had been much simplified, and it had been agreed to use State Party reports required within IHR (2005).

APSED (2010) did indeed provide a clear framework for shared responsibility but he assured representatives that it was flexible about implementation. Experience had shown that a funded and resourced workplan was one means to implement the strategy and develop the core capacities required by IHR (2005). He was aware of the diversity of Member States in the Region and recognized that IHR (2005) had legal status, but said that national plans could be developed to deal with national needs.

In response to the Representative of Japan he said that IHR communications was a system owned by all. Improvements were always welcome and the Regional Office and WHO Headquarters were seeking ways to do that.

He agreed with the comments about keeping influenza as a priority and using the lessons learnt from the recent pandemic, especially as influenza A virus subtype H5 was circulating in the Region. Response and preparedness needed to be continued in an effective and sustainable way. APSED (2010) incorporated the lessons learnt from the pandemic.

In response to the comments from the Representatives of the Philippines and Japan, he said that he expected that the monitoring and evaluation, human resource implications and bolstering of national IHR focal point capacity would be dealt with during implementation. Although the points raised by the Representative of the United States of America about surveillance of sexually transmitted infections and second-line medicines for resistant tuberculosis were not a primary focus of APSED (2010), he said that coordination with other groups would be encouraged in order to seek synergy during implementation.

In reply to the Representative of Hong Kong (China), he said that APSED (2010) was the product of a series of country and regional consultations. Comments about the name had been made and the consensus had been to keep the name as there was existing momentum and recognition of the name, and the focus on emerging diseases remained a priority.

The Assistant Director-General for Health Security and Environment observed that many Member States in the Region had participated in the discussions on the International Health Regulations. IHR (2005), a legally binding framework, had been adopted as a result of recent epidemics and at their heart laid the drive to build capacities. APSED (2005) had been a very successful attempt at working biregionally to increase capacities, and provided an excellent example of cooperation between regions.

Delegates at the World Health Assembly had mentioned, like the Representative of the Lao People's Democratic Republic, that the reporting tool was complicated. He agreed and said that work was under way at WHO Headquarters to simplify it.

In reply to the Representative of France, he said that IHR (2005) did allow direct reporting in principle. The Secretariat had corresponded with governments to that effect, pointing out that the matter was a question of dialogue between the territories and their capitals to resolve.

He applauded the historical lesson given by the Representative of Samoa: historical experiences remained germane and should not be forgotten.

He also greatly appreciated the intervention by the Representative of the Solomon Islands that, having experienced an event, countries needed to learn the lessons and respond appropriately.

The DIRECTOR, PROGRAMME MANAGEMENT, acknowledged the thoughtful comments and support. The large number of interventions attested to the attention paid by Member States to the subject. Governments in the Region had made substantial investments in building the core capacities for early detection and response required by IHR (2005) and had used the framework of APSED (2005) to support that process. An improvement in the draft revised Strategy was the clearer direction on the need to prepare for a broader range of public health events, many of which were regrettably common in the Region.

The Regional Director had pointed out in his report that he had reorganized several units into a new Division of Health Security and Emergencies in order to provide better support to Member States.

He assured representatives that APSED (2010) was not too prescriptive; it had been designed to assist all parties in developing and maintaining the core capacities required to implement IHR (2005). Some countries had those capacities, whereas others did not, but APSED (2010) had been designed to suit any situation.

He too commented on the name, citing examples that testified to the value of retaining a name when the image and branding were good and well recognized.

In the absence of further comments, The CHAIRPERSON asked the Rapporteurs to prepare an appropriate draft resolution for consideration later in the session.

3. VACCINE-PREVENTABLE DISEASES: Item 15 of the Agenda (document WPR/RC61/10)

The DIRECTOR, PROGRAMME MANAGEMENT, presented the report on the status of measles elimination, hepatitis B control and poliomyelitis eradication in the Western Pacific Region to the Regional Committee and pointed out that 2010 marked the 10th anniversary of the declaration of the Region's poliomyelitis-free status. However, it was necessary to maintain sufficient immunization coverage to keep the Region free of poliomyelitis and to protect against the threats posed by importation, which had been seen on the Region's borders in 2010.

The Region was also continuing to work hard to meet the twin goals of measles elimination and hepatitis B control by 2012. However, there were potential problems. While many Member States were achieving those goals, slower progress elsewhere was placing measles elimination and hepatitis B control at risk.

The report before the Committee summarized the steps that needed to be taken, including a renewed commitment to measles elimination and sustained high routine immunization coverage. There was also a need to establish a verification process for those countries and areas that had eliminated measles.

The Regional Committee was invited to consider the report and to discuss the next steps to achieve its important goals.

Mrs MATALAVEA (Samoa) said that the recently completed national Demographic Health Survey 2009 had revealed shockingly low immunization coverage in Samoa and had been a salutary jolt for the Samoan Health Service. Her Government was therefore keen to sign up to the 2012 regional goals for measles and hepatitis B. The Samoan Government was committed to increasing vaccination coverage for all preventable diseases and intended to make school-entry immunization a legal requirement.

Dr SHARMA (Fiji) said that for many years the Fijian immunization budget had never reflected the actual investment made in purchasing vaccines to protect school-age children against nine serious infectious diseases. The current budget represented about one-third of the actual cost of purchasing vaccines each year, and Fiji had therefore been obliged to stagger its payments to the Vaccine Independence Initiative, a revolving fund managed by the United Nations Children's Fund (UNICEF). A further investment of US\$ 5.8 million was required to introduce pneumococcal vaccine, rotavirus vaccine and human papillomavirus vaccine, with a view to achieving Millennium Development Goals 4 and 5. In the past five years, a number of new vaccines had been developed and introduced in many developed countries, but Fiji would only be able to afford them if its vaccine budget was tripled in 2011 or alternative sources of funding could be found. The opportunity to introduce new vaccines into its childhood immunization schedule would reduce outpatient visits, hospital admissions and maternal and child mortality. His Government was grateful to WHO, the Australian Agency for International Development and the Secretariat of the Pacific Community for their assistance in dealing with the recent outbreak of typhoid in Fiji, and to WHO for providing 86 000 doses of pandemic influenza H1N1 (2009) vaccine. Fiji still had 800 vials of vaccine left, which it would willingly share with other Pacific island countries.

Dr KIRITION (Kiribati) said that Kiribati was grappling with a number of problems that prevented it from guaranteeing timely vaccination and achieving high immunization rates. Administration of the first dose of hepatitis B vaccine within 24 hours of birth required vaccines to be readily available at all maternity units, which meant that working refrigerators had to be on hand. Maintaining the cold chain in Kiribati had not been an easy task. His Government acknowledged WHO's role in working with vaccine refrigerator manufacturers and had noted with interest that cheaper, more efficient WHO-certified solar vaccine refrigerators had now come on to the market. Kiribati did not have a wild poliovirus importation preparedness plan and would therefore seek guidance from WHO and other technical partners. His Government supported the drive to strengthen the national case-based surveillance and reporting system for suspected measles and acute flaccid paralysis (AFP) cases, and hoped for increased cooperation with WHO and UNICEF in that area. With regard

to vaccine supply, logistics and security, his Government particularly wished to stress the important role played by UNICEF through its Pacific Vaccine Independence Initiative.

Dr VILLAVERDE (Philippines) said that his Government was committed to eliminating measles, controlling hepatitis B and rubella and maintaining poliomyelitis-free status. The Philippines strongly supported the establishment of a national measles elimination verification committee.

Dr XIA (China) said that, following the commitments agreed at the fifty-sixth session of the Regional Committee, China had developed national measles eradication plans for the period 2006–2012 and 2010–2012, and had proceeded to implement them. The Chinese Government also endorsed WHO's strategy for hepatitis B control, in line with which it had emphasized routine immunization of newborns and supplementary immunization of children under 15, in addition to guaranteeing an uninterrupted cold chain for hepatitis B vaccine. WHO's efforts to maintain the poliomyelitis-free status of the Region were to be commended. China had eradicated poliomyelitis back in 2000, since when it had strengthened national AFP surveillance. The Western Pacific Region's poliomyelitis-free status was currently precarious, and hence his Government hoped to acquire more information about poliovirus strains outside the Region.

Ms LANGIDRIK (Republic of the Marshall Islands) said that the Marshall Islands had already reached the goal of reducing chronic hepatitis B infection to less than 2% among 5-year-olds. Since the 2003 measles outbreak in the Marshall Islands, an immunization programme had continued to reach remote communities with a view to protecting children and adults alike. The national immunization coverage rate was 89% in 2009 and was increasing annually by 2%. In 2008, the Ministry of Health had added rotavirus, haemophilus influenza and pneumococcal vaccines to its immunization programme, and in the same year had started to target girls and women aged 13–26 with HPV vaccine. In urban centres, 100% of babies born in hospital received the first dose of hepatitis B vaccine within 24 hours of birth, along with Bacilli Calmette Guérin (BCG). However, 2% of babies born in the remoter islands did not receive hepatitis B vaccine within 24 hours of birth. Her Government acknowledged the continued support of the United States Centers for Disease Control and Prevention, WHO and UNICEF in its immunization programme.

Dr KHALIFAH ISMAIL (Brunei Darussalam) said that Brunei Darussalam had a long history of providing free childhood immunization. In 2007, the national health authorities had rescheduled the second dose of MMR vaccine to cover children aged 3. His Government was currently reviewing its national immunization programme with a view to enhancing it. Clinical laboratory surveillance of poliomyelitis, measles, mumps and rubella was already in place, and the long-established national certification committee for certification of poliomyelitis eradication was continuing to maintain AFP surveillance at the highest possible standard. National guidelines had been developed on preparedness and response to importation of wild poliovirus, and a poliomyelitis outbreak contingency plan had been put in place. Measles test kits had been made widely available and the authorities intended to complete a hepatitis B serosurveillance study in schoolchildren within the next two years. The Government was also promoting vaccine supply, immunization and injection safety, the use of disposable syringes and the safe disposal of used needles and syringes, in addition to ensuring good hygiene standards through the use of closed sewage systems.

Dr SONG (Republic of Korea) said that reported measles cases in the Region were at an all-time low, with an estimated 27 countries and areas likely to achieve the 2012 milestone goal for chronic

hepatitis B infection. The Region had remained free of poliomyelitis in spite of the persistent risk of poliovirus importation. However, several countries were still experiencing measles outbreaks and others were unlikely to achieve the 2012 goal for hepatitis B. Furthermore, the recent poliomyelitis outbreak in Tajikistan, which had reported no cases of poliomyelitis since 1997, had served as a stark reminder that the Western Pacific Region could be the next destination for the virus. The concerted efforts of all Member States to accomplish their vaccine-preventable disease goals and the strengthening of each Member State's national immunization programme could not be overemphasized. Her Government supported the use of measles elimination resources to facilitate control of rubella and prevention of congenital rubella syndrome. The Republic of Korea also supported the establishment of a verification process for measles elimination and encouraged other Member States with sufficient data on hepatitis B control to obtain certification from the Regional Office.

Ms HSU (United States of America) thanked the Secretariat for its detailed and comprehensive account of the inroads that Member States had made in addressing a number of vaccine-preventable diseases, together with the information on immunization coverage, importation-related outbreaks and the political, financial and human resource support for current efforts. The Region had made good progress on interrupting endemic measles and rubella virus transmission, and the Regional Director should promote the establishment of a verification process. Her Government urged Member States to remain vigilant about consistent measles vaccination in order to prevent the international spread of the disease, and also to integrate rubella-containing vaccines into their national immunization programmes. Further progress was required on hepatitis B control. Specific technical support should be offered to countries that were having difficulty in achieving high coverage using a timely birth dose and/or three-dose hepatitis B vaccine.

The number of cases of wild poliovirus importation in countries neighbouring the Western Pacific Region was a source of major concern, and accordingly her Government urged the Regional Office and Member States to sustain political and programmatic commitment to poliomyelitis eradication, measles elimination and hepatitis B control, in particular by emphasizing the need for high-quality AFP, measles and rubella surveillance, continuing oral poliovirus vaccine and measles supplementary immunization activities.

Finally, her Government suggested that the Interagency Coordination Committee (ICC) should be strengthened as a platform for resource mobilization to address resource shortfalls and to encourage the development of sustainable national programmes. The ICC should meet more regularly, thereby ensuring participation by decision-makers, closer coordination, significantly enhanced contributions by governments, and greater private sector and bilateral agency partnership with priority countries.

Dr TAKEI (Japan) said that his Government had established prevention guidelines with a view to eliminating measles by 2012, and had achieved promising results. The regular vaccination rate of 1-year-old infants had been maintained at 90% and the number of measles cases had decreased from 11 015 in 2008 to 741 in 2009, or by 93%. Approximately 500 measles cases were forecast in Japan in 2010. Once measles had been eliminated, a verification process would need to be established. However, given that many Member States in the Western Pacific Region were still reporting measles cases, the emphasis should continue to be on elimination as opposed to verification.

Ms BENNETT (Australia) commended the Organization's contribution to the elimination of measles, control of hepatitis B and defence of the Region's polio-free status. Member States had to prioritize scarce

resources to strengthen routine and supplementary immunization. Australia supported the setting up of a verification process for measles elimination in Member States, which should build, where possible, on existing structures. Immunization coverage in the Region should be maintained and increased. It would be good to see a move from supplementary immunization services to routine services, especially to maintain measles coverage; she sought guidance from WHO on how to achieve that. In Australia, rubella and congenital rubella syndrome were effectively being controlled through routine, free childhood immunization.

Mr VU SINH NAM (Viet Nam) supported the WHO milestone on vaccine-preventable diseases. The Expanded Programme on Immunization had started in Viet Nam in 1981, covering the entire country from 1985. In 1987, four new vaccines had been added for children in high-risk areas: hepatitis B; Japanese encephalitis; cholera; and typhoid. In 2003, with support from GAVI and WHO, hepatitis B vaccination had been extended to the entire country. In June 2010, HiB vaccine was added to the EPI for infants. Eleven vaccines were used in the Programme, with over 95% coverage in the target population. Viet Nam supported the recommendations of the 19th meeting of the Technical Advisory Group on Immunization and Vaccine-preventable Diseases in the Western Pacific Region. There was careful monitoring of the impact of supplementary immunization activities, targeting children up to the age of 5 in order to determine whether measles virus circulation has been interrupted, and intensive surveillance in the wake of supplementary immunization. Such activities should be extended to young adults, prioritizing those such as students, and health and community workers who were at highest risk from measles.

Dr LAM (China [Macao]) reported that, before 1990, the immunization programme in Macao (China) had included only a single dose of measles vaccine for children at 9 months of age. There had been intermittent outbreaks of the disease at that time. Subsequently, second and third doses had been added, and, in 2005, a two-dose schedule—measles, mumps and rubella at 12 months and at 18 months—had been introduced. Surveillance had been strengthened in line with regional guidelines. Macao had now reached the elimination target for measles in terms of incidence and surveillance quality. It fully supported the regional target on measles elimination and would continue to work towards that goal.

Dr FAKAKOVIKAETAU (Tonga) commended the continuing efforts of the Regional Office to eradicate vaccine-preventable diseases from the Region. In the 1960s, BCG vaccination had been introduced in her country, even although TB had not been a threat to the population. Incidence of TB was still very low, at as few as five new cases per year in the previous three years. Poliomyelitis had been absent for more than 10 years, and also measles, although in 2002 there had been an outbreak of rubella. Tonga had had a particularly high rate of hepatitis B in its population, with prevalence as high as 18%. Since the introduction of hepatitis B vaccine, however, the prevalence of hepatitis B surface antigen in children had dropped to 0.8% by 2005. Like the Representative of the Marshall Islands, she believed that hepatitis B had been eliminated among young people. Finally, her Government wished to make a plea that affordable new meningococcal vaccine should be made available to benefit countries throughout the Region.

At the invitation of the CHAIRPERSON, a statement was made to the Committee by the representative of Rotary International.

The meeting rose at 17.20 p.m

